

Life, Critical Illness, and Disability Claims

An update on

- **CII examination and**
- **AMUS Professional Qualification**

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Agenda

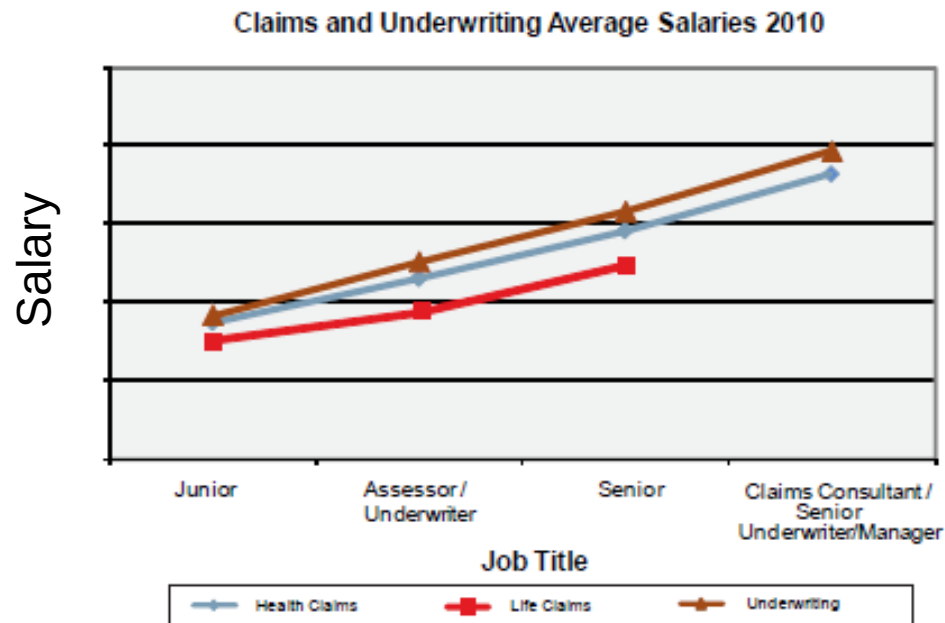
- **Background**
- New coursebook
- New exam
- New professional qualification

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- Life and Disability Claims (Subject 556) introduced by CII in 2002.

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- Life and Disability Claims (Subject 556) introduced by CII in 2002.
- 50 to 60 candidates sit the exam each year
- 69 people have gone on to be awarded the AMUS Diploma in Life and Disability Claims
 - Exam passes in Claims, Life Assurance Practice, and Insurance Law
 - Relevant vocational experience

Why?!

- Expand knowledge
 - Current role
 - Other aspects of claims
- Improve performance and customer service in current role
- Enhance opportunities for other roles and career development
- Improve the image of claims assessors as a professional body
- AMUS 'Towers' award for outstanding candidates

Why the Change?

- To keep it relevant
- To encourage participation
- To fit with new CII qualification structure
- To take on board feedback
- To keep in line with wider educational framework

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Revised Coursebook

Out Now!

- Re-written in 2012 with the help of:

- Ann Marie Dec – Scor
- Alex Isted – Munich Re
- Mary Grindle – Scor
- Tim Knaust – Swiss Re
- Stuart Lewis – Met Life
- Gill Nurse - Aviva
- Stephanie McMahon – Swiss Re
- ...and others!

Thank you!

Revised Coursebook

Out now!

- Removal of less relevant or duplicated text that is not relevant to most assessors and rarely questioned e.g.
 - Long term care
 - Actuarial aspects
- Updated writing style and learning tools
 - Web links
 - Activities
 - Self tests
- Available electronically!
- Reference tool for day to day claims assessment

DRAFT**B Checks to be made on initial notification****B1 In force status**

For any claim to be considered the policy must be in force at the time of the claim

For any claim to be considered the policy must be in force at the time of the claim, i.e. the happening of the insured event. The assessor must check the commencement and expiry dates of the policy, and ensure that there has been no interruption to the cover during the term of the policy that may affect the validity of the claim.

Most claim events have a readily identifiable date of claim, e.g. date of death under a life policy, or date of heart attack under a critical illness policy. Occasionally there may be some debate as to what date the insured event actually occurred and insurers may have to take a pragmatic approach based on the information that is available, along with input from legal or medical advisers.

**Think**

How would you determine the date of claim in the following scenarios?

- A man goes missing whilst out at sea in a small vessel. His body is washed up three weeks later.
- A woman with progressively worsening eyesight over the last three years submits a claim for blindness.

What if the policy expired during the three weeks that the man was missing, or six months before the blind claimant contacted you?

The post-mortem may give an estimation of when the man died. The woman may have had regular check-ups that can provide an objective assessment of the deterioration in her vision.

B1A Commencement date

When notified of a claim it is necessary to ensure that the event for which a claim is being made occurred on or after the date the policy commenced. Whilst this may be straightforward for the majority of cases, it is important for the claims assessor to distinguish between the commencement date of the policy and the date the insurer agreed terms for the policy. These dates are not always one and the same. The policy may not actually start for some months after the policy has been accepted by the underwriters, and it is not uncommon for claims to arise in the intervening period or there to be some uncertainty as to exactly when the cover commenced.

The date the policy commenced is an important fact for the claims assessor. It determines the date from which any claim can be considered, and also usually marks the point up to when the applicant should disclose any change to their health or circumstances.

**Example 2.3**

An applicant for insurance completes an application form on 1 May. On 15 May the underwriter accepts the application on the insurer's standard terms, and a letter of acceptance is issued that day. A direct debit mandate is already in place and the applicant requests the policy starts from 1 June.

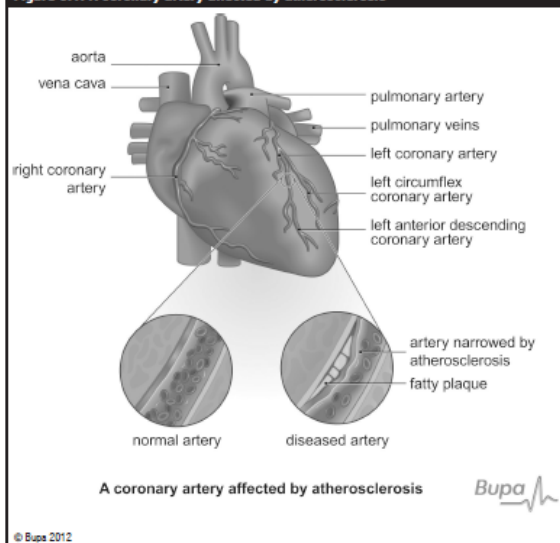
In this case although terms were offered on 15 May the policy does not commence until 1 June and no claim can be considered for events prior to that date. The proposer is also likely to be under a duty to disclose any changes to their health or relevant risk factors up until 1 June.

In some cases an applicant may ask for a commencement date in the past. Say, for example, in the above case the applicant had asked for a commencement date of 1 May. If an insured event occurred on 10 May the insurer would not be liable for this as it occurred prior to the date the underwriter accepted the risk.

Insurers should ensure they make clear to all applicants the continuing duty of disclosure and what this means. If there is a significant delay between the date of application and the policy commencement it is prudent to remind the applicant of the ongoing duty. Failure to highlight the nature and timescales of the duty of disclosure often compromises an insurer's position when contested cases are adjudicated by the Financial Ombudsman Service (FOS). In addition at least one insurer now waives the life assured's duty to make a disclosure of changes between completing the proposal and the policy going into force.

Smaller vessels branch off these arteries to cover the surface area of the heart and supply all areas of heart muscle with blood, see figure 9.1.

Figure 9.1: A coronary artery affected by atherosclerosis



It is important that arteries and veins allow blood to flow freely, especially those serving the muscles of the heart. Over time fatty deposits can build up within these blood vessels; a process known as atherosclerosis. These deposits can form plaques (blockages) that impede the blood flow to the heart. This process can result in angina, heart attack, or even death.

Atherosclerosis of the coronary arteries and its various manifestations may be known as ischaemic heart disease, coronary heart disease or coronary artery disease.

Ischaemia develops in situations where the oxygen demands of the heart muscle exceed the capacity of the coronary arteries to meet those requirements. This usually arises as a result of atherosclerosis in the coronary arteries, but it can also arise in the presence of normal coronary arteries (for example, when the arteries go in to spasm, or the flow of blood is impeded by impaired functioning of the walls of the heart).

Ischaemic heart disease may begin in early life but does not usually manifest clinically until late middle age. Asymptomatic or undetected coronary disease is extremely common; at least 50% of men in the UK are thought to have some degree of coronary atheroma.

Useful website

For more information about the effects, prevention, treatment and cause of heart disease in the UK go to www.bhf.org.uk

Ischaemic heart disease is linked to a number of risk factors including:

- cigarette smoking;
- high blood pressure;
- high cholesterol levels;

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Revised Exam

- **Old Exam (556)**
 - April and October
 - 3 hours
 - Continuous Assessment 10%
 - 8 short compulsory questions
48 marks
 - 1 long compulsory question
50 marks
 - 3 (from 5) long questions
102 marks
- **New Exam (P62)**
 - April and October
 - 3 hours
 - Continuous Assessment 10%
 - 14 short(ish) compulsory questions
140 marks
 - 2 (from 3) long questions
60 marks

Revised Exam

- Written by assessors for assessors
- Reviewed and agreed by a panel of assessors
- Sat by a 'scrutineer' prior to the exam being finalised
- Marking scheme approved – and precise
- Individual papers marked with random sampling, and moderation meetings to set pass mark

Example Short Question

15 marks

- Explain
 - the nature, symptoms and treatment of ulcerative colitis; and
 - under what circumstances this condition might give rise to a valid IP or TPD claim.

Example Long Question

30 marks

As part of the assessment of a CI claim for breast cancer it is noted that a policyholder had become aware of a lump in her breast prior to applying for the policy on the 28th April 2012.

No adverse disclosures were made on the application form, and the policy commenced on the 1st May 2012 at standard rates.

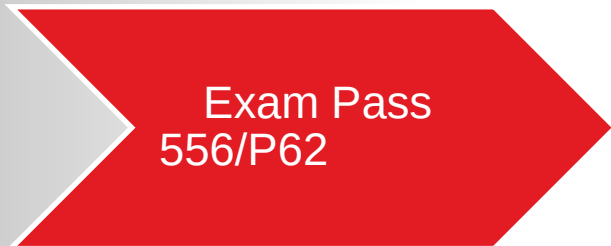
She first consulted her GP in respect of the breast lump on 7th May 2012.

Identify what action an assessor would need to take in order to determine whether non disclosure has arisen, and if so explain what the consequences would be.

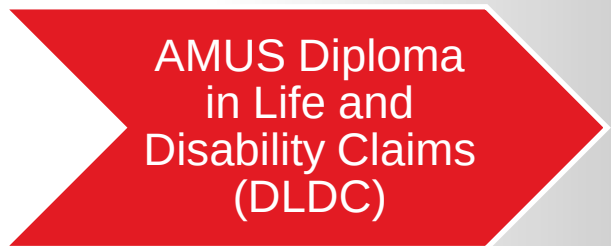
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Qualification Structure



Exam Pass
556/P62



AMUS Diploma
in Life and
Disability Claims
(DLDC)

- 3 exam passes
- 2-4 years experience
- Reference

Revised Qualification Structure



- 1 exam pass (P62/555)
 - 1 year experience
 - Reference and CPD
- 3 exam passes
 - 2-4 years experience
 - Reference

Recap

- Lots of changes
- New and improved!
- Relevant and valued qualification
- Up to date, more user friendly learning materials
- Enhanced opportunity to obtain professional qualification
 - Diploma
 - Certificate



Thank you for your attention.

Questions?